

SUPERVISED VOCATIONAL QUALIFICATION DIPLOMA - EXAMINATION

Session: Revision Mock Set 09	Duration: 3 hours
Specialty: Psychotherapy	Coefficient: 04
Eliminatory mark: < 8/20	Training reference: VTI-AEDA examination style

COMPREHENSIVE VOCATIONAL PAPER - SOLUTION / MARKING GUIDE PRACTICE SET 09

Revision material based on the supplied VTI-AEDA 2025 examination format and the supplied psychotherapy course recordings. Not an official examination paper.

How to use this marking guide

For revision, first attempt the paper without notes. Then compare not only the answer letter but the clinical cue that makes the answer correct. For legal/ethical items, the guide follows the concepts emphasized by the supplied course/examination materials; in real practice, applicable Cameroon law, institutional policy and the current professional code must also be checked.

PART ONE: PSYCHOPATHOLOGY - ANSWERS

1. Wendy, a 19-year-old shop manager, presents with abnormally elevated or irritable mood with increased energy, reduced need for sleep, pressured speech, grandiosity, racing thoughts and risky activity. Which diagnosis or clinical concept best fits this presentation?

- A. Brief psychotic disorder
- B. Substance use disorder
- C. Bulimia nervosa

D. Manic episode

Answer: D - Manic episode. A manic episode combines abnormal mood elevation/irritability with increased energy and characteristic activation symptoms.

Memory aid: DIG FAST: Distractible, Indiscretion, Grandiosity, Flight of ideas, Activity, Sleep less, Talkative.

2. A therapist notes that Tina has a history of a manic episode, often alternating with periods of depression. Which condition should be considered first from the options below?

- A. Bipolar disorder**
- B. Manic episode
- C. Borderline personality disorder
- D. Stimulant intoxication

Answer: A - Bipolar disorder. The defining feature of bipolar I disorder is a manic episode; depressive episodes are common but not required for that diagnosis.

Memory aid: BI-polar = mood can move toward two poles; mania is the key flag.

3. During assessment, Owen reports persistent psychotic illness with delusions, hallucinations and/or disorganized speech or behavior, often with social or occupational dysfunction. Which of the following is the BEST match?

- A. Brief psychotic disorder
- B. Stimulant intoxication
- C. Schizophrenia**
- D. Binge-eating disorder

Answer: C - Schizophrenia. Schizophrenia is a chronic psychotic disorder; diagnosis depends on characteristic symptoms, duration and functional impact, not on a laboratory test alone.

Memory aid: 3D psychosis: Delusions, perceptual Disturbance, Disorganization.

4. A therapist notes that Amina has sudden onset of delusions, hallucinations or disorganized speech/behavior lasting at least a day but less than a month, with return toward baseline. Which condition should be considered first from the options below?

- A. Obsessive-compulsive personality disorder (OCPD)

B. Brief psychotic disorder

- C. Stimulant intoxication
- D. Borderline personality disorder

Answer: B - Brief psychotic disorder. The very short duration and return toward premorbid functioning distinguish brief psychotic disorder from longer psychotic disorders.

Memory aid: BRIEF = psychosis Below one month.

5. During assessment, Carlos reports significantly low body weight, intense fear of weight gain and restriction without recurrent binge-purge episodes. Which of the following is the BEST match?

- A. Obsessive-compulsive personality disorder (OCPD)
- B. Brief psychotic disorder
- C. Specific phobia

D. Anorexia nervosa, restricting type

Answer: D - Anorexia nervosa, restricting type. Anorexia nervosa requires significantly low weight plus fear of gaining weight or persistent behavior interfering with weight gain and body-image disturbance.

Memory aid: ANOREXIA: restriction + low weight + fear of weight gain.

6. A therapist notes that Lucas has significantly low body weight with anorexia features plus recurrent binge-eating or purging. Which condition should be considered first from the options below?

- A. Anorexia nervosa, binge-eating/purging type**
- B. Opioid intoxication
- C. Specific phobia
- D. Substance use disorder

Answer: A - Anorexia nervosa, binge-eating/purging type. The patient meets anorexia criteria including low weight, but also has recurrent binge/purge behavior.

Memory aid: Low weight first: if anorexia criteria are met, binge/purge does not automatically mean bulimia.

7. A therapist notes that Uma has recurrent binge eating followed by compensatory behaviors such as vomiting, laxatives or excessive exercise, without the persistently low weight required for anorexia. Which condition should be considered first from the options below?

- A. Bulimia nervosa**
- B. Stimulant intoxication
- C. Manic episode
- D. Obsessive-compulsive personality disorder (OCPD)

Answer: A - Bulimia nervosa. Bulimia is defined by binge episodes and inappropriate compensation; weight may be normal or above/below normal but not at the markedly low anorexia threshold.

Memory aid: BULIMIA = Binge + Undo.

8. During assessment, Lucas reports recurrent binge eating with loss of control and distress but without regular compensatory purging or fasting. Which of the following is the BEST match?

- A. Stimulant intoxication
- B. Post-traumatic stress disorder (PTSD)
- C. Binge-eating disorder**
- D. Specific phobia

Answer: C - Binge-eating disorder. Binge-eating disorder differs from bulimia because recurrent compensatory behaviors are absent.

Memory aid: BED = Binge, no regular Erasing behavior.

9. Joel, a 32-year-old civil servant, presents with a problematic pattern of substance use causing clinically significant impairment or distress, including loss of control, risky use, social impairment or tolerance/withdrawal. Which diagnosis or clinical concept best fits this presentation?

- A. Obsessive-compulsive disorder (OCD)
- B. Substance use disorder**

- C. Schizophrenia
- D. Stimulant intoxication

Answer: B - Substance use disorder. Substance use disorder is diagnosed from a maladaptive pattern and consequences, not simply from having used a substance.

Memory aid: 4C idea: Control loss, Consequences, Compulsion/craving, Continued use.

10. Fidel, a 40-year-old driver, presents with initial euphoria followed by drowsiness or apathy, with slowed functioning and often constricted pupils. Which diagnosis or clinical concept best fits this presentation?

- A. Brief psychotic disorder
- B. Opioid intoxication**
- C. Anorexia nervosa, binge-eating/purging type
- D. Binge-eating disorder

Answer: B - Opioid intoxication. Opioids typically produce analgesia, euphoria followed by sedation and physiologic slowing.

Memory aid: OPIOID = Often Pinpoint pupils, Inward/sedated, Opioid drowsiness.

SECTION B: TRUE/FALSE - EXPLANATIONS

1. TRUE. A panic attack can occur in disorders other than panic disorder.

A panic attack is a symptom episode and can occur across several disorders; panic disorder requires a recurrent pattern plus ongoing concern or behavior change.

2. FALSE. Obsessions are repetitive behaviors, whereas compulsions are intrusive thoughts.

The relationship is reversed: obsessions are intrusive thoughts/urges/images; compulsions are repetitive behaviors or mental acts.

3. TRUE. Trauma exposure can increase risk for depression, anxiety and substance-related problems.

Trauma can be followed by several forms of psychopathology, not only PTSD.

4. FALSE. A single laboratory test confirms schizophrenia.

Schizophrenia is diagnosed clinically from symptoms, course, duration and impairment while ruling out other causes.

5. TRUE. In major depression, loss of interest or pleasure is called anhedonia.

Anhedonia is a core depressive symptom.

SECTION C: MODEL SHORT ANSWERS

1. Differentiate OCD from obsessive-compulsive personality disorder. (2mks)

- OCD involves obsessions and/or compulsions causing distress/time burden.
- OCPD is a pervasive pattern of perfectionism, orderliness and control; obsessions/compulsions are not required.

Memory aid: OCD = obsessions/compulsions; OCPD = order/control/personality.

2. Give two signs that suggest psychosis. (2mks)

- Examples: delusions, hallucinations, markedly disorganized speech/behavior, impaired reality testing.

Memory aid: 3D: Delusions, perceptual Disturbance, Disorganization.

3. Why must suicide risk be assessed in a severely depressed client? (2mks)

- Depression may involve hopelessness and suicidal ideation; assessment identifies immediacy of risk and need for protective intervention/referral.

Memory aid: Ask directly about thoughts, plan, intent, means and protective factors.

4. State two differences between GAD and a specific phobia. (2mks)

- GAD: diffuse worry across multiple domains; specific phobia: circumscribed fear of a particular object/situation.
- GAD is persistent generalized worry; phobia is triggered by the feared stimulus and avoidance.

Memory aid: General vs specific.

5. What does DSM refer to in psychopathology? (2mks)

- Diagnostic and Statistical Manual of Mental Disorders.
- A classification system used to describe diagnostic criteria; it supports but does not replace clinical assessment.

Memory aid: *DSM = Diagnostic + Statistical + Manual.*

PART TWO: THEORIES/TECHNIQUES - ANSWERS

1. Which psychotherapy concept is most accurately described by the following: the therapist's emotional reactions to a client, especially reactions influenced by the therapist's own history or unresolved issues?

- A. Cognitive restructuring
- B. Adlerian therapy
- C. Countertransference**
- D. Existential therapy

Answer: C - Countertransference. Countertransference concerns the therapist's responses to the client and must be monitored through self-awareness and supervision.

Memory aid: COUNTER = counselor side of the relational reaction.

2. Which psychotherapy concept is most accurately described by the following: therapy emphasizing unconscious conflict, early relationships, defenses, transference, resistance and insight?

- A. Rational Emotive Behavior Therapy (REBT)
- B. Psychoanalytic therapy**
- C. Cognitive restructuring
- D. Cognitive behavioral therapy (CBT)

Answer: B - Psychoanalytic therapy. Psychoanalytic approaches seek insight into unconscious patterns and how early relationships repeat in present life and therapy.

Memory aid: FIRT: Free association, Interpretation, Resistance, Transference.

3. Which psychotherapy concept is most accurately described by the following: a goal-directed, socially embedded approach emphasizing conscious choice, responsibility, lifestyle and movement from inferiority toward useful belonging?

- A. Empty-chair technique
- B. Existential therapy
- C. Adlerian therapy**
- D. Interpretation

Answer: C - Adlerian therapy. Adlerian therapy views behavior as purposeful and socially related, with emphasis on goals, choice and responsibility. This is explicitly reflected in the opening lecture of the fused audio.

Memory aid: GSCR: Goal-directed, Social relatedness, Conscious choice, Responsibility.

4. In a session with Zachary, the counselor emphasizes a non-directive relationship based on accurate empathy, unconditional positive regard and congruence. This BEST illustrates:

- A. Person-centered therapy**
- B. Rational emotive imagery
- C. Extinction
- D. Adlerian therapy

Answer: A - Person-centered therapy. Rogers' person-centered therapy relies on core facilitative conditions rather than therapist-imposed solutions.

Memory aid: EUC: Empathy, Unconditional positive regard, Congruence.

5. In a session with Grace, the counselor emphasizes a structured approach linking thoughts, emotions and behaviors and using cognitive and behavioral change strategies. This BEST illustrates:

- A. Rational emotive imagery
- B. Negative reinforcement
- C. Cognitive behavioral therapy (CBT)**
- D. Social skills training

Answer: C - Cognitive behavioral therapy (CBT). CBT targets maladaptive thinking and behavior through collaborative, structured methods such as cognitive restructuring and behavioral experiments.

Memory aid: TEA: Thoughts - Emotions - Actions.

6. Which psychotherapy concept is most accurately described by the following: identifying inaccurate or dysfunctional thoughts, examining evidence and developing more balanced alternatives?

- A. **Cognitive restructuring**
- B. Psychoanalytic therapy
- C. Negative reinforcement
- D. Classical conditioning

Answer: A - Cognitive restructuring. Cognitive restructuring is a core CBT method for testing and modifying distorted or unhelpful cognitions.

Memory aid: *Catch it, Check it, Change it.*

7. A therapist working with Wendy uses an intervention characterized by Albert Ellis' approach that identifies irrational beliefs between events and emotional consequences and actively disputes those beliefs. Which approach or technique is being used?

- A. Free association
- B. Extinction
- C. **Rational Emotive Behavior Therapy (REBT)**
- D. Classical conditioning

Answer: C - Rational Emotive Behavior Therapy (REBT). REBT uses the ABC model and disputation to replace rigid irrational beliefs with more rational alternatives.

Memory aid: *ABCDE: Activating event, Belief, Consequence, Dispute, Effective new belief.*

8. In a session with Tina, the counselor emphasizes an REBT imagery exercise in which the client vividly imagines a difficult event, experiences the emotion, then practices changing unhealthy emotion by changing beliefs. This BEST illustrates:

- A. **Rational emotive imagery**
- B. Flooding
- C. In vivo exposure
- D. Systematic desensitization

Answer: A - Rational emotive imagery. Rational emotive imagery is an experiential REBT technique for rehearsing healthier emotional responses through belief change.

Memory aid: *Imagine - feel - dispute - re-feel.*

9. Which psychotherapy concept is most accurately described by the following: a here-and-now experiential approach emphasizing awareness, contact, unfinished business and integration?

- A. Transference
- B. Extinction
- C. **Gestalt therapy**
- D. Systematic desensitization

Answer: C - Gestalt therapy. Gestalt therapy focuses on present awareness and experiments that help clients own and integrate experience.

Memory aid: *Gestalt = awareness in the NOW.*

10. A therapist working with Amina uses an intervention characterized by the client speaks to an imagined person or part of self placed in an empty chair, often switching roles to deepen awareness. Which approach or technique is being used?

- A. **Empty-chair technique**
- B. Interpretation
- C. Adlerian therapy
- D. Classical conditioning

Answer: A - Empty-chair technique. The empty-chair experiment is associated with Gestalt work on unfinished business, polarities and present-moment experience.

Memory aid: *Empty chair = make the absent relationship present.*

11. In a session with Lucas, the counselor emphasizes the client re-enacts a dream in the present and may speak as different people, objects or elements of the dream. This BEST illustrates:

- A. Dialectical Behavior Therapy (DBT)
- B. Cognitive restructuring
- C. Progressive muscle relaxation

D. Gestalt dream work

Answer: D - Gestalt dream work. Rather than imposing a fixed interpretation, Gestalt dream work invites the client to own and experience parts of the dream.

Memory aid: *Dream it, become it, speak it.*

12. In a session with Tina, the counselor emphasizes turning back onto oneself what one wants to do to another, or doing to oneself what one wishes another would do. This BEST illustrates:

- A. In vivo exposure
- B. Resistance

C. Retroflexion

- D. Social skills training

Answer: C - Retroflexion. Retroflexion is a Gestalt contact-boundary disturbance directed back toward the self.

Memory aid: *RETRO = return action back to self.*

13. Which psychotherapy concept is most accurately described by the following: an approach emphasizing freedom, responsibility, choice, meaning, authenticity, isolation and awareness of mortality?

- A. Extinction
- B. Free association
- C. Empty-chair technique

D. Existential therapy

Answer: D - Existential therapy. Existential therapy helps clients face givens of existence and make authentic choices rather than simply trying to eliminate all anxiety.

Memory aid: *FMRC: Freedom, Meaning, Responsibility, Choice.*

14. A therapist working with Owen uses an intervention characterized by learning by association between stimuli so that a previously neutral cue comes to elicit a response. Which approach or technique is being used?

- A. Cognitive behavioral therapy (CBT)
- B. Negative reinforcement
- C. Cognitive restructuring

D. Classical conditioning

Answer: D - Classical conditioning. Classical conditioning explains learned associations such as fear conditioning and underlies some exposure-based principles.

Memory aid: *Classical = cue-to-cue association.*

15. A therapist working with Victor uses an intervention characterized by behavior is shaped by its consequences, including reinforcement and punishment. Which approach or technique is being used?

- A. Social skills training
- B. Extinction
- C. Operant conditioning
- D. Social modeling

Answer: C - Operant conditioning. Operant conditioning focuses on consequences that increase or decrease behavior.

Memory aid: *Operant = behavior OPERATES, consequence follows.*

SECTION B: TRUE/FALSE - EXPLANATIONS

1. **TRUE.** Accurate empathy, unconditional positive regard and congruence are core person-centered conditions.

These are the classic Rogers conditions emphasized in person-centered therapy.

2. TRUE. Albert Ellis is the major proponent of Rational Emotive Behavior Therapy.

Ellis developed REBT.

3. TRUE. Sigmund Freud is the foundational proponent of psychoanalytic therapy.

Freud developed psychoanalysis.

4. FALSE. Free association requires the therapist to censor embarrassing client material.

The client is invited to say what comes to mind without censorship.

5. FALSE. Transference refers mainly to the therapist transferring personal feelings onto the client.

That description is closer to countertransference; transference is the client carrying past relationship patterns into the therapeutic relationship.

SECTION C: MODEL SHORT ANSWERS

1. State one technique specific to psychoanalytic work. (2mks)

- Free association, interpretation, analysis of resistance, transference analysis or dream analysis - any one.

Memory aid: *FIRT*.

2. Explain systematic desensitization in three steps. (2mks)

- Teach relaxation/coping response.

- Build a graded fear hierarchy.

- While relaxed, expose progressively from easier to harder items until anxiety reduces.

Memory aid: *RHE*.

3. Distinguish flooding from graded exposure. (1mks)

- Flooding begins with high-intensity feared material and sustains exposure.

- Graded exposure moves stepwise from lower to higher fear levels.

Memory aid: *Flooding = deep end; graded = ladder.*

PART THREE: ETHICAL/LEGAL - ANSWERS

1. During supervision, Lucas describes maintaining accurate, timely, secure and clinically appropriate client records. Which concept should guide the discussion?

- A. Record keeping**
- B. Nonmaleficence
- C. Boundary crossing
- D. Termination and referral

Answer: A - Record keeping. Records support continuity of care, accountability and legal/ethical duties; access must be controlled and retention rules followed.

Memory aid: ACTS: Accurate, Current, Timely, Secure.

2. A counselor asks which principle or professional concept is defined by ending therapy responsibly with preparation, continuity planning and referral when needed rather than abandoning the client. The BEST answer is:

- A. Termination and referral**
- B. Malpractice
- C. Professional competence
- D. Nonmaleficence

Answer: A - Termination and referral. Ethical termination includes adequate notice, discussion, referrals or continuity plans when appropriate.

Memory aid: PLAN: Prepare, Link/referral, Address feelings, Next steps.

3. During supervision, Fidel describes using ethical codes, consultation/supervision and appropriate reporting procedures when credible serious misconduct is observed. Which concept should guide the discussion?

- A. Nonmaleficence
- B. Bartering
- C. Ethical response to colleague misconduct**
- D. Mandated reporting

Answer: C - Ethical response to colleague misconduct. Serious ethical violations should not simply be ignored; follow code, organizational and legal procedures while protecting clients and due process.

Memory aid: C-A-R-E: Consult code, Assess facts, Report appropriately, Ensure client safety.

4. A counselor asks which principle or professional concept is defined by protecting privacy, competence, informed consent, identity, emergency planning and data security when using digital or remote services. The BEST answer is:

- A. Autonomy
- B. Technology ethics**
- C. Bartering
- D. Nonmaleficence

Answer: B - Technology ethics. Technology changes the medium, not the professional duty. Security, consent and contingency planning remain essential.

Memory aid: SPACE: Security, Privacy, Access/identity, Consent, Emergency plan.

5. In professional practice, a situation calls for not automatically disclosing the entire file on receipt of a subpoena; consult applicable law, the client/attorney and professional guidance, and disclose only as authorized or legally required. Which ethical/legal concept is MOST directly involved?

- A. Dual or multiple relationship
- B. Record keeping
- C. Beneficence
- D. Subpoena response**

Answer: D - Subpoena response. A subpoena and a court order are not always identical. The safest course-based principle is not to reflexively release all records; seek proper legal/ethical guidance and protect confidentiality to the extent permitted.

Memory aid: STOP: See legal guidance, Tell/consult client, Only required information, Protect confidentiality.

6. A counselor asks which principle or professional concept is defined by respecting the client's right to make informed choices about treatment. The BEST answer is:

- A. Informed consent
- B. Confidentiality
- C. Autonomy**
- D. Malpractice

Answer: C - Autonomy. Autonomy supports informed choice, consent and respect for the client's self-determination.

Memory aid: A = Allow informed choice.

7. A counselor asks which principle or professional concept is defined by acting to promote the client's welfare and benefit. The BEST answer is:

- A. Termination and referral
- B. Confidentiality
- C. Autonomy
- D. Beneficence**

Answer: D - Beneficence. Beneficence asks the professional to actively contribute to the client's well-being.

Memory aid: BENEFIT = beneficence.

8. During supervision, Yvette describes avoiding or minimizing harm. Which concept should guide the discussion?

- A. Bartering
- B. Confidentiality
- C. Informed consent
- D. Nonmaleficence**

Answer: D - Nonmaleficence. Nonmaleficence is the duty to do no harm and to avoid exposing clients to unnecessary risk.

Memory aid: NON-maleficence = NO harm.

9. In professional practice, a situation calls for fair and equitable distribution of services, benefits, burdens and opportunities. Which ethical/legal concept is MOST directly involved?

- A. Bartering
- B. Mandated reporting
- C. Justice**
- D. Duty to warn/protect

Answer: C - Justice. Justice concerns fairness and equitable treatment, including access and resource allocation.

Memory aid: Justice = fairness.

10. During supervision, Fidel describes truthfulness and honest communication with clients. Which concept should guide the discussion?

- A. Duty to warn/protect
- B. Subpoena response
- C. Record keeping
- D. Veracity**

Answer: D - Veracity. Veracity requires accurate, truthful information rather than deception.

Memory aid: VERACITY = verifiable truth.

11. During supervision, Wendy describes keeping commitments, maintaining trust and being faithful to professional responsibilities. Which concept should guide the discussion?

- A. Informed consent
- B. Fidelity**
- C. Justice

D. Nonmaleficence

Answer: B - Fidelity. Fidelity concerns loyalty, promises and trust within professional relationships.

Memory aid: *Fidelity = faithful to promises.*

12. In professional practice, a situation calls for an ongoing process in which the client receives understandable information about services, risks, benefits, alternatives, limits of confidentiality and choice. Which ethical/legal concept is MOST directly involved?

A. Informed consent

B. Autonomy

C. Justice

D. Ethical response to colleague misconduct

Answer: A - Informed consent. Informed consent is not merely a signature; it is an ongoing communication process supporting voluntary, informed participation.

Memory aid: *BRALC: Benefits, Risks, Alternatives, Limits, Choice.*

13. In professional practice, a situation calls for the professional duty to protect information learned in the therapeutic relationship, subject to recognized legal and ethical exceptions. Which ethical/legal concept is MOST directly involved?

A. Confidentiality

B. Mandated reporting

C. Privileged communication

D. Record keeping

Answer: A - Confidentiality. Confidentiality protects client information but is not absolute; limits should be explained early.

Memory aid: *Confidentiality = keep private, know the exceptions.*

14. In professional practice, a situation calls for the client's broader right to control access to self, body and personal information. Which ethical/legal concept is MOST directly involved?

A. Justice

B. Malpractice

C. Privacy

D. Nonmaleficence

Answer: C - Privacy. Privacy is the client's right to be left alone or control access, whereas confidentiality is the professional's duty regarding information received.

Memory aid: *Privacy belongs to the person; confidentiality binds the professional.*

15. During supervision, Joel describes a legal protection that may prevent compelled disclosure of certain therapy communications in judicial proceedings, depending on jurisdiction. Which concept should guide the discussion?

A. Privileged communication

B. Nonmaleficence

C. Boundary violation

D. Fidelity

Answer: A - Privileged communication. Privilege is a legal evidentiary concept and is distinct from the broader ethical duty of confidentiality.

Memory aid: *Privilege = courtroom/legal shield.*

SECTION B: MODEL SHORT ANSWERS

1. What is a boundary violation? (2mks)

- A serious harmful or exploitative breach of therapeutic boundaries.

Memory aid: *Violation = harm/exploitation.*

2. State two technology-related ethical concerns in teletherapy. (1mks)

- Examples: data security/privacy, informed consent, identity verification, competence with technology, emergency location/plan, jurisdiction.

Memory aid: SPACE.

3. How should a therapist respond to a subpoena for records? (1mks)

- Do not reflexively release the entire file.
- Consult applicable law/professional guidance and the client/attorney; protect confidentiality and disclose only what is authorized or legally required.

Memory aid: STOP.

4. What is the principle of justice? (1mks)

- Fair and equitable treatment and distribution of benefits, burdens and resources.

Memory aid: Justice = fairness.

SECTION C: MODEL SHORT ESSAYS

1. What does duty to warn/protect mean in therapy? (2mks)

- When a serious credible threat to an identifiable person is present, the therapist may have to take reasonable protective action according to applicable law and ethics, even though confidentiality is normally protected.

Memory aid: TIP: Threat, Identifiable person, Protect.

2. Define mandated reporting. (2mks)

- A legal/professional requirement to report specified situations such as suspected abuse to designated authorities; it is a limit to confidentiality.

Memory aid: Mandatory = must report when the law says so.

3. State two ways a therapist should manage sexual or romantic feelings toward a client. (1mks)

- Acknowledge and monitor the feelings rather than act on them.
- Seek supervision/consultation and strengthen boundaries; refer if objectivity/competence cannot be maintained.

Memory aid: FEEL, do not ACT: notice, supervise, protect boundaries.

PART FOUR: CULTURE / GROUP / COUNSELING PROCESS / TREATMENT PLAN - ANSWERS

1. A trainee observes a group emphasizing mutual support, shared experience, acceptance and coping among people facing similar problems. This is an example of:

- A. Ethnocentrism
- B. Support group**
- C. Performing stage
- D. Assessment

Answer: B - Support group. Support groups reduce isolation and offer mutual aid through shared experience.

Memory aid: *SUPPORT = Similar people Understanding and Providing Peer help.*

2. In counseling practice, Tina encounters a situation involving a structured group focused on teaching information, understanding a condition and coping knowledge. Which concept BEST applies?

- A. Paraphrasing
- B. Psychoeducational group**
- C. Support group
- D. Active listening

Answer: B - Psychoeducational group. Psychoeducational groups primarily teach information and understanding; they may include discussion and practice.

Memory aid: *EDU = education is central.*

3. Which counseling/group/cultural concept is BEST described by: a group focused on learning and practicing specific behavioral or interpersonal skills?

- A. Skills-development group**
- B. Group role
- C. Reflection of feeling
- D. Psychoeducational group

Answer: A - Skills-development group. Skills groups emphasize rehearsal, practice and feedback rather than information alone.

Memory aid: *SKILLS = Show, Keep practicing, Improve, Learn, feedback.*

4. In counseling practice, Nadine encounters a situation involving a structured group using cognitive and behavioral methods such as thought monitoring, skills practice and behavioral exercises. Which concept BEST applies?

- A. Psychoeducational group
- B. Performing stage
- C. Cognitive-behavioral group**
- D. Group confidentiality

Answer: C - Cognitive-behavioral group. CBT groups combine structured learning with practice and feedback around thoughts and behaviors.

Memory aid: *Group CBT = Think + Do together.*

5. Which counseling/group/cultural concept is BEST described by: a recurrent functional position adopted by a member, such as scapegoat, monopolizer, peacemaker or spokesperson?

- A. Professional interpreter use
- B. Open-ended questioning
- C. Group role**
- D. Rapport building

Answer: C - Group role. Group roles describe patterned ways members participate and influence group dynamics.

Memory aid: *Role = repeated position in the group.*

6. Which counseling/group/cultural concept is BEST described by: an agreement that members protect information disclosed by other members, while the leader explains that peer confidentiality cannot be guaranteed in the same way as professional confidentiality?

- A. Termination and follow-up
- B. Storming stage
- C. Group confidentiality**
- D. Performing stage

Answer: C - Group confidentiality. Confidentiality is essential in groups, but leaders should explain realistic limits because members are not all licensed professionals.

Memory aid: *What is said in group should stay in group - with limits explained.*

7. A trainee observes early group stage characterized by orientation, uncertainty, dependence and testing. This is an example of:

- A. Open-ended questioning
- B. Cultural imposition
- C. Forming stage**
- D. Performing stage

Answer: C - Forming stage. Forming is the beginning stage in which members learn the purpose, structure and safety of the group.

Memory aid: *FSNPA: Forming, Storming, Norming, Performing, Adjourning.*

8. A trainee observes group stage in which conflict, resistance, competition or emotional testing becomes more visible. This is an example of:

- A. Performing stage
- B. Reflection of feeling
- C. Open-ended questioning
- D. Storming stage**

Answer: D - Storming stage. Storming often brings disagreement and challenges to roles or leadership.

Memory aid: *Storm = conflict weather.*

9. In counseling practice, Owen encounters a situation involving group stage in which cohesion, trust, norms and cooperation consolidate. Which concept BEST applies?

- A. Therapeutic confrontation
- B. Cultural imposition
- C. Psychoeducational group
- D. Norming stage**

Answer: D - Norming stage. Norming is marked by clearer expectations and stronger cohesion.

Memory aid: *Norming = norms settle.*

10. In counseling practice, Esther encounters a situation involving mature working stage in which members use the group productively toward goals. Which concept BEST applies?

- A. Performing stage**
- B. Reflection of feeling
- C. Storming stage
- D. Termination and follow-up

Answer: A - Performing stage. Performing is the task-focused, therapeutically productive phase.

Memory aid: *Performing = productive work.*

11. In counseling practice, Martha encounters a situation involving ending stage involving review, separation, consolidation and transfer of learning. Which concept BEST applies?

- A. Adjourning stage**
- B. Open-ended questioning
- C. Ethnocentrism
- D. Group role

Answer: A - Adjourning stage. Adjourning addresses termination and helps members carry gains beyond the group.

Memory aid: *Adjourning = goodbye and generalize gains.*

12. In counseling practice, Uma encounters a situation involving creating safety, trust, empathy and collaborative engagement at the beginning of counseling. Which concept BEST applies?

- A. Case formulation
- B. Norming stage
- C. Rapport building**
- D. Skills-development group

Answer: C - Rapport building. Rapport supports disclosure and collaboration; it begins early but continues throughout treatment.

Memory aid: *RAPPORT starts with Respect, Attention, Presence.*

13. Which counseling/group/cultural concept is BEST described by: systematically exploring the client's problems, symptoms, context, history, strengths, risk and functioning?

- A. Ethnocentrism
- B. Treatment plan
- C. Performing stage
- D. Assessment**

Answer: D - Assessment. Assessment precedes or accompanies formulation and goal setting; it includes both problems and resources.

Memory aid: *HRSF: History, Risk, Symptoms, Functioning.*

14. In counseling practice, Amina encounters a situation involving collaboratively defining client-centered outcomes that are realistic and observable. Which concept BEST applies?

- A. Norming stage
- B. Open-ended questioning
- C. Skills-development group
- D. Goal setting**

Answer: D - Goal setting. Goals are stronger when the client participates and the goals are specific enough to evaluate.

Memory aid: *SMART: Specific, Measurable, Achievable, Realistic/Relevant, Time-bound.*

15. A trainee observes implementing agreed therapeutic strategies and monitoring the client's response. This is an example of:

- A. Assessment
- B. Cognitive-behavioral group
- C. Intervention stage**
- D. Forming stage

Answer: C - Intervention stage. Intervention follows assessment and goal definition and should be linked to the case formulation and treatment plan.

Memory aid: *Assess -> Goal -> Intervene -> Review.*

SECTION B: MODEL SHORT ESSAYS

1. Outline two benefits of group therapy. **(4mks)**

- Reduces isolation/universality: members learn they are not alone.
- Provides mutual support and feedback.
- Allows interpersonal learning, modeling and practice.
- Can be efficient and increase access - any two well explained.

Memory aid: *U-S-M: Universality, Support, Modeling.*

2. State two characteristics of a culturally competent therapist. **(4mks)**

- Self-awareness of own values/biases.
- Knowledge of different cultural contexts and humility about what is not known.

- Adaptable communication/clinical skills.
- Willingness to seek cultural encounters/consultation and avoid imposition - any two explained.

Memory aid: AKSE.

3. What is the counseling process? (2mks)

- A collaborative sequence that typically includes engagement/rapport, assessment, goal setting, intervention, review, and termination/follow-up.

Memory aid: RAGIRT: Rapport, Assessment, Goals, Intervention, Review, Termination.

SUPERVISED VOCATIONAL QUALIFICATION DIPLOMA - EXAMINATION

Session: Revision Mock Set 09	Duration: 1hr 30mins
Specialty: Psychotherapy	Coefficient: 04
Eliminatory mark: < 14/20	Training reference: VTI-AEDA examination style

SIMULATION OF REAL WORKING SITUATION PAPER - SOLUTION / MARKING GUIDE PRACTICE SET 09

Revision material based on the supplied VTI-AEDA 2025 examination format and the supplied psychotherapy course recordings. Not an official examination paper.

DETAILED MODEL ANSWERS - SIMULATION OF REAL WORKING SITUATIONS

SECTION I

1. Panic with avoidance - anchor: Panic disorder with agoraphobic avoidance - CBT and exposure

Chantal, 63, has recurrent sudden episodes of racing heart, sweating, breathlessness and fear of dying. After several attacks in public places, she now avoids buses, markets and long queues and only goes out with a companion.

Model response / marking points (25 marks):

1. Assess attack features, medical/substance causes, frequency, anticipatory anxiety and avoidance.
2. Explain the panic cycle and misinterpretation of bodily sensations.
3. Teach realistic reappraisal of sensations.
4. Use interoceptive exposure where appropriate to feared bodily sensations.
5. Build graded in vivo exposure to avoided settings.
6. Reduce safety behaviors gradually.
7. Use breathing skills carefully as coping, not as proof that anxiety is dangerous.
8. Track predictions and actual outcomes.
9. Assess risk and comorbidity.
10. Plan relapse prevention.

Suggested mark allocation: Award marks for correct formulation/approach, risk and ethical priorities, logical session structure, appropriate technique application, collaboration/homework/follow-up, and professional communication. Credit clinically sound alternatives when clearly justified. Do not award full marks for naming a theory without showing how it is applied to the facts of the vignette.

Memory aid: Panic: BODY sensation -> catastrophic THOUGHT -> FEAR -> avoidance; exposure breaks the loop.

2. OCD contamination and washing - anchor: CBT with Exposure and Response Prevention (ERP)

Dieudonne, 22, spends several hours each day washing hands and cleaning after intrusive fears of contamination. he knows the rituals are excessive but believes that not washing could cause serious harm to family members.

Model response / marking points (25 marks):

1. Clarify obsessions, compulsions, triggers, time burden and insight.
2. Explain how rituals reduce anxiety briefly and thereby maintain OCD.
3. Develop a graded exposure hierarchy collaboratively.
4. Expose to feared contamination cues in planned steps.
5. Prevent or delay the washing ritual during/after exposure.
6. Remain long enough for new learning rather than immediate escape.
7. Address inflated responsibility/catastrophic beliefs with cognitive methods as needed.
8. Do not provide repeated reassurance that becomes part of the compulsion.

9. Use homework and repeated practice.
10. Monitor distress, safety and progress.

Suggested mark allocation: Award marks for correct formulation/approach, risk and ethical priorities, logical session structure, appropriate technique application, collaboration/homework/follow-up, and professional communication. Credit clinically sound alternatives when clearly justified. Do not award full marks for naming a theory without showing how it is applied to the facts of the vignette.

Memory aid: ERP = Expose + Resist ritual + Practice.

SECTION II

1. Social anxiety and skills practice - anchor: Social anxiety - CBT, graded exposure and social skills training if deficits are present

Estelle, 29, avoids speaking in meetings and meeting new people because of fear of blushing, stumbling over words and being judged as incompetent. This is beginning to limit career and friendships.

Model response / marking points (25 marks):

1. Assess feared situations, predictions, avoidance and actual skill deficits.
2. Explain self-focused attention and avoidance/safety behaviors.
3. Identify negative predictions about judgment.
4. Build graded social/performance exposures.
5. Use behavioral experiments and reduce safety behaviors.
6. If skill deficits exist, use instruction, modeling, role play, rehearsal and feedback.
7. Practice attention shifting toward the interaction.
8. Set homework in real social situations.
9. Review outcomes against predictions.
10. Plan maintenance and relapse prevention.

Suggested mark allocation: Award marks for correct formulation/approach, risk and ethical priorities, logical session structure, appropriate technique application, collaboration/homework/follow-up, and professional communication. Credit clinically sound alternatives when clearly justified. Do not award full marks for naming a theory without showing how it is applied to the facts of the vignette.

Memory aid: IMRF for skills: Instruction, Modeling, Rehearsal, Feedback.

2. Trauma symptoms and stabilization - anchor: PTSD/trauma presentation - safety, grounding and stabilization before intensive processing when dissociation is high

After a violent incident, Franck, 36, experiences nightmares, intrusive memories, avoidance, hypervigilance and episodes of feeling detached or unreal. he becomes overwhelmed when asked to describe the event in detail.

Model response / marking points (25 marks):

1. Assess trauma symptoms, duration, functioning, suicide/self-harm risk, substance use and current safety.
2. Recognize intrusion, avoidance, negative mood/cognition and arousal; assess dissociation.
3. Explain trauma responses and normalize without minimizing.
4. Use grounding/orientation to present, paced breathing and stabilization skills.
5. Establish a window of tolerable arousal and stop/modify processing if dissociation increases.
6. Identify supports and coping resources.
7. Collaboratively set goals and consent for later trauma processing.
8. Use evidence-based trauma-focused treatment only when sufficiently stable and within competence.
9. Coordinate psychiatric/medical care where needed.
10. End with reorientation and a clear safety/follow-up plan.

Suggested mark allocation: Award marks for correct formulation/approach, risk and ethical priorities, logical session structure, appropriate technique application, collaboration/homework/follow-up, and professional communication. Credit clinically sound alternatives when clearly justified. Do not award full marks for naming a theory without showing how it is applied to the facts of the

vignette.

Memory aid: SAFER: *Safety, Assess, Frame/psychoeducate, Establish grounding, Review readiness.*